TAMAYUZ 10X

OPERATIONAL EXCELLENCE · WHAT THE RESEARCH SAYS

Many health systems show doctors their own performance data.

In a review of many randomized trials, recommended practice rose by about 6 percentage points on average, compared with clinicians who did not get their data.

But the effect varied widely. Here is what five studies, published from 2021 to 2026, show.

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THE IDEA

What is audit and feedback?

Measuring the performance of a doctor or team over a set period, then showing them the results.

A percentage point is the difference between two percentages. Going from 50% to 56% is a rise of six percentage points.

THE COCHRANE REVIEW

The review pooled 292 randomized trials

6.2

percentage points: the average rise in recommended practice, in 177 trials comparing clinicians who got their data with those who did not.

2.7

points or less: the improvement in half of the results. At least a quarter showed no improvement. This figure is calculated differently, so do not compare it directly with the average.

Improvement was larger among doctors who started from low performance.

ANTIBIOTIC PRESCRIBING

Primary care doctors prescribed fewer antibiotics

11%

fewer antibiotics prescribed by doctors who received their data, in 21 trials.

23%

fewer antibiotics started without need, in 16 trials.

Both are relative reductions, measured against doctors who did not receive the data. They are not percentage points, so they cannot be compared directly with the 6.2 points on the previous slide. The researchers noted that results varied widely across trials.

MIXED RESULTS

Not everything improved

In Ontario, Canada, the polyp removal rate rose by similar amounts among colonoscopy doctors who got a report and those who did not. But lower performers who got a report improved more than lower performers who did not.

In seven pediatric emergency departments, recording of vital signs improved, while over the same period speed of care got worse.

In 201 bladder cancer surgery centers, recording of surgical findings improved where reports were sent, but early cancer recurrence did not differ.

WHAT MADE THE DATA MORE USEFUL

When was the effect larger?

When the data were about the doctor's own practice and compared it with top peers or a set benchmark.

When someone from the same workplace whom the doctor knew presented the data.

When a plan with specific steps came with the report.

Most of these findings came from comparing results across trials, and the researchers said they need confirmation.

THE AUTHOR'S READING OF THE STUDIES

What can leaders do?

1

Choose an important measure where performance is still far from the goal.

2

Show each person their own numbers, have someone they know discuss them, and agree on a specific plan.

3

Watch other measures that could get worse, not only the one you target.

All three steps are the author's inference from the studies. No study tested them together.

> **Three years from now, should performance reports in your organization be documents people forget, or a regular review of each person's numbers with a colleague they know?**

SOURCES AND LIMITS

What this evidence can and cannot tell us

In 39% of the Cochrane trials, flaws in design or conduct could make the result inaccurate (high risk of bias).

The review measured what health professionals did, not patient health.

None of the five study abstracts mentions a trial in an Arab country. Applying the idea to other sectors is an untested inference.

Editorial disclaimer: The views and analyses presented in this article represent the author's professional interpretation based on the cited sources and do not necessarily represent the official positions of the organizations mentioned. For the original, complete and most current information, please consult the primary sources and official documents listed in the references.

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